Vet. App. No. 19-0320

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IN THE UNITED STATES COURT OF APPEALS
FOR VETERANS CLAIMS

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DONALD R. FULTON,
Appellant,

v.

DONALD R. FULTON,
ROBERT L. WILKIE,
Secretary of Veterans Affairs,
Appellee.

v.

ROBERT L. WILKIE,
ON APPEAL FROM THE
BOARD OF VETERANS' APPEALS
Secretary of Veterans Affairs,
Appellee.

BRIEF OF APPELLEE
SECRETARY OF VETERANS AFFAIRS
ON APPEAL FROM THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

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## II. STATEMENT OF THE CASE

### Nature of the Case

Appellant, Donald Fulton, appeals the November 8, 2018, decision of the Board of Veterans' Appeals (Board) that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation, and peripheral vascular disease, claimed as ischemic heart disease. [Record Before the Agency (R.) at 9 (4-12)].

The Board also granted entitlement to a separate, compensable rating for left foot ulcer. [R. at 4]. This favorable finding cannot be disturbed. *See Medrano v. Nicholson*, 21 Vet.App. 165, 170 (2007) ("The Court is not permitted to reverse findings of fact favorable to a claimant made by the Board pursuant to its statutory authority.").

The Board also remanded Appellant's claim of entitlement to a total disability rating based on individual unemployability due to service-connected disability. [R. at 4]. The Court is without jurisdiction over this issue. *See Breeden v. Principi*, 17 Vet.App. 475, 478 (2004) (per curiam order) (stating that a Board remand decision is not a final decision over which this Court has jurisdiction); 38 U.S.C. §§ 7252(a), 7266(a).

### Statement of Facts and Procedural History

Appellant served on active duty from August 1964 to August 1967, including service in the Republic of Vietnam. [R. at 1292].

## B. Statement of Facts and Procedural History

Appellant served on active duty from August 1964 to August 1967, including service in the Republic of Vietnam. R. at 1292.

Appellant's July 1964 entrance medical examination indicated a sitting blood pressure reading of 110/70. [R. at 2631 (2630-31)]. The examiner noted that Appellant's heart and vascular system were clinically normal. [R. at 2630].

Appellant's June 1967 separation medical examination indicated a sitting blood pressure reading of 122/82. [R. at 2629 (2628-29)]. The examiner noted that Appellant's heart and vascular system were clinically normal. [R. at 2628].

In December 2004, thirty-seven years after his separation from service, Appellant underwent a cardiolite stress test at the Armstrong County Memorial Hospital. [R. at 2332-33]. The test report indicated that Appellant was admitted to the hospital with atrial fibrillation but "no prior history of cardiac problems." [R. at 2332]. The report stated that Appellant had "no history of hypertension, diabetes mellitus, or hypercholesterolemia." [R. at 2332]. The report noted a resting blood pressure reading of 132/91 and "at peak stress" of 172/112. [R. at 2332]. The test results "did not show any changes suggestive of ischemia." [R. at 2333]. Appellant's VA medical records indicate an onset of benign hypertension in 2004. [R. at 2433 (2432-34)].

In July 2010, Appellant was granted service connection for posttraumatic stress disorder and type 2 diabetes mellitus associated with herbicide exposure. [R. at 2472 (2472-75)]. At that time, Appellant submitted a request for service connection for ischemic heart disease and bilateral neuropathy in feet due to diabetes. [R. at 2452-53]. In December 2010, Appellant was afforded a VA examination. [R. at 2407-09]. The examination listed diagnoses of atrial

## SUMMARY OF THE ARGUMENT

The issue of entitlement to service connection for hypertension was not reasonably raised by the record because the medical evidence indicated that Appellant's

If the factfinder's account of the evidence is plausible in light of the record viewed in its entirety, the reviewing court may not reverse it even though convinced that had it been sitting as the trier of fact, it would have weighed the evidence differently. *Id.* (citations omitted). Where there are two permissible views of the evidence, the factfinder's choice between them cannot be clearly erroneous. *Id.* (citations omitted).

## ARGUMENT

### I. STANDARD OF REVIEW

"Whether an issue is reasonably raised by the record is essentially a question of fact, subject to the 'clearly erroneous' standard of review." *Lynch v. Wilkie*, 30 Vet.App. 296, 304 (2018) (citations omitted). "A finding is 'clearly erroneous' when although there is evidence to support it, the reviewing court on the entire evidence is left with the definite and firm conviction that a mistake has been committed." *Gilbert v. Derwinski*, 1 Vet.App. 49, 52 (1990) (citation omitted).

### II. THE BOARD DID NOT FAIL TO ADDRESS A REASONABLY RAISED THEORY.

The Board is required to consider all issues raised either by the claimant or by the evidence of record. *Robinson v. Peake*, 21 Vet.App. 545, 552 (2008) (citations omitted). However, this obligation "does not require the Board to assume the impossible task of inventing and rejecting every conceivable argument in order to produce a valid decision." *Id.* at 553. "[T]he Board is not required to conjure up issues that were not raised by the appellant." *Brannon v. West*, 12 Vet.App. 32, 35 (1998) (citation omitted); *see also Sondel v. Brown*, 6 Vet.App. 218, 220 (1994) (when issue is not reasonably raised, Board is not required to "conduct an exercise in prognostication").

Appellant argues that the Board failed to address a claim for hypertension that was reasonably raised by the evidence in Appellant's case. [Appellant's Brief (App. Br.) at 8-15]. Appellant's argument is flawed for at least four reasons: (1) Appellant relies upon medical authorities that were not part of the record before the Board, (2) Appellant's theory is contrary to the medical evidence of record, (3) an isolated blood pressure reading is not clinically significant, and (4) the Board is

not required to analyze all medical test results in the record that might be elevated, but not abnormal, to conjure up issues that were not raised by a claimant.

First, Appellant improperly relies upon medical authorities that were not part of the record before the Board. Appellant's brief cites to the Merck Manual for threshold blood pressure readings indicative of normal blood pressure, prehypertension, and Stage 1 hypertension. [App. Br. at 3, 11]. Appellant's brief also relies on a report from the National Academy of Sciences (NAS) regarding veterans and Agent Orange. [App. Br. at 11-12]. The NAS report and the Merck Manual were not before the Board in November 2018. [R. at 4-12]. Therefore, this Court should decline to entertain Appellant's argument relying on these documents. *See Euzebio v. Wilkie*, __ Vet.App. __, No. 17-2879, 2019 U.S. App. Vet. Claims LEXIS 1476, at *9–14 (Aug. 22, 2019) (finding NAS reports were not constructively before the Board); 38 U.S.C. § 7252(b); *Rogozinski v. Derwinski*, 1 Vet.App. 19, 19 (1990) (holding that review in this Court shall be on the record of proceedings before the Secretary and the Board); *Bell v. Derwinski*, 2 Vet.App. 611, 613 (1992) (holding that VA has constructive possession of documents "if they are within the Secretary's control and could reasonably be expected to be a part of the [record]").

Appellant's argument that hypertension was reasonably raised by the record depends on the premise that the NAS report and the Merck Manual were constructively before the Board. [App. Br. at 11-12]. However, following Appellant's brief, this Court held in *Euzebio* that a NAS report was not

- Appellant's July 1964 entrance medical examination indicated that Appellant's heart and vascular system were clinically normal [R. at 2630];
- Appellant's June 1967 separation medical examination indicated that Appellant's heart and vascular system were clinically normal [R. at 2628];
- Appellant's private medical report from 2004 states that Appellant had no history of hypertension [R. at 2332]; and
- Appellant's VA medical records indicate an onset of benign hypertension in 2004, thirty-seven years after Appellant's separation from service [R. at 2433].

The evidence of record indicated that Appellant's heart and vascular system were clinically normal and that there was an onset of benign hypertension in 2004, thirty-seven years after Appellant's separation from service. The theory that Appellant's hypertension was service connected was not reasonably raised by the record.

Third, an isolated blood pressure reading is not clinically significant. It is VA's policy to accept a hypertension diagnosis only on the basis of "readings taken two or more times [per day] on at least three different days" so as to avoid prematurely diagnosing hypertension on the basis of lone, possibly unrepresentative measurements. *See* 38 C.F.R. § 4104, DC 7101, Note (1); *accord Gill v. Shinseki*, 26 Vet.App. 386, 390 (2013) ("Note (1) . . . states that hypertension must be confirmed by at least two blood pressure readings taken over several days"). This is consistent with the examiner's conclusion that Appellant's heart and vascular system were clinically normal in June 1967 and the

practice in *Colvin v. Derwinski*, 1 Vet.App. 171, 175 (1991), and *Hime v. McDonald*, 28 Vet.App. 1, 5-6 (2016).

Given the evidence before the Board in this case, a claim of service connection for hypertension was not reasonably raised by the record.

## V. CONCLUSION

For the foregoing reasons, the Secretary respectfully requests that the Court affirm the November 8, 2018, decision of the Board that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation and peripheral vascular disease, claimed as ischemic heart disease.

## CONCLUSION

For the foregoing reasons, the Secretary respectfully requests that the Court affirm the November 8, 2018, decision of the Board that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation and peripheral vascular disease, claimed as ischemic heart disease.

Respectfully submitted,

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November 18, 2019

## V. CONCLUSION

For the foregoing reasons, the Secretary respectfully requests that the Court affirm the November 8, 2018, decision of the Board that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation and peripheral vascular disease, claimed as ischemic heart disease.

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## CERTIFICATE OF SERVICE

I certify that on November 18, 2019, I filed the foregoing Brief of Appellee Secretary of Veterans Affairs with the Clerk of the Court and served it on counsel for the other party by the Court's electronic filing system.

s/ Mark J. Hamel
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